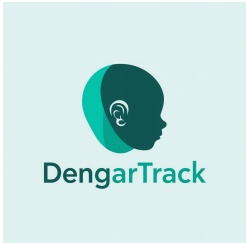


CORE SELLING POINT

From bedside screening to national programme visibility

DengarTrack is a Malaysia-ready digital workflow platform for Universal Newborn Hearing Screening (UNHS). It replaces fragmented paper cards, Excel files, WhatsApp coordination, and manual reports with one connected, auditable workflow.



"DengarTrack is not just a screening app. It is a complete UNHS digital care pathway: QR bedside screening, offline capture, follow-up tracking, audit documentation, and KKM-ready reporting in one scalable platform."



One bedside entry flows into operational action, governance evidence, and programme reporting.

97.1%

Malaysia household internet acc...

Local cloud

AWS Malaysia Region available

2024+

PDPA amendments strengthen g...

The Best Conference Angle

Angle	What to emphasize
Workflow transformation	The project digitises the whole UNHS journey, not only the screening form. One cot-side action becomes a record, follow-up task, audit trail, and report.
LTFU reduction	The follow-up queue makes RUJUK cases visible, prioritised, and accountable instead of being scattered across paper, Excel, or informal messaging.
Malaysia-ready design	BM default, Android-first, offline-capable, QR-based, and built around public hospital realities where WiFi and staff workload can be inconsistent.
Scalable backend foundation	FastAPI, PostgreSQL, JWT/RBAC, audit logs, report APIs, and structured modules make future integration with i-Jejak, HIS, FHIR/HL7, reminders, and multi-hospital rollout practical.

What Makes This Strong

Component	Current project value	Conference message
Flutter mobile app	QR baby lookup, bedside screening entry, BM/EN toggle, secure token login, offline pending screening storage.	Fast and practical for ward use.
Coordinator workflow	Hospital dashboard, follow-up queue, urgency filtering, status actions, monthly reports, Excel export.	Turns RUJUK cases into tracked action.
UNHS dashboard	All-hospital aggregate view, programme summary, recent audit monitoring, no individual baby records.	Programme oversight without overexposing patient data.
MOH / KKM view	National aggregate dashboard, trends, hospital breakdown, policy attention signals, aggregate-only access.	Supports population-level monitoring and policy decisions.
Backend + data model	Role-based API enforcement, PostgreSQL schema, audit logs, anonymised baby system IDs, report endpoints.	The foundation is the product's real strength.

Why It Fits Malaysia's Digital Landscape

Malaysia dynamic	How DengarTrack answers it
High digital readiness	Malaysia already has very high internet access, and hospital staff are familiar with mobile-first workflows. A QR + Android-first app is easier to adopt than a heavy desktop-only system.
Healthcare digitalisation is still uneven	MOH's Health White Paper notes that EMR has not been fully rolled out and databases/registries still have limited linkages. DengarTrack addresses one focused clinical workflow while remaining integration-ready.
Data governance pressure is increasing	PDPA 2024 changes and public sensitivity around health data make RBAC, audit logs, anonymised IDs, and aggregate-only ministry views important selling points.
Local cloud/data residency is now more realistic	AWS Malaysia Region supports lower-latency, in-country cloud deployment for future pilot or rollout phases.

The One-Line Pitch

English: DengarTrack turns newborn hearing screening from a fragmented paper-based task into a complete digital care pathway: QR bedside screening, offline capture, follow-up tracking, audit documentation, and KKM-ready reporting.

BM: DengarTrack bukan sekadar aplikasi saringan, tetapi platform workflow digital UNHS yang menghubungkan saringan di katil bayi kepada susulan, dokumentasi audit, pemantauan hospital, dan pelaporan KKM dalam satu sistem yang scalable.

Recommended Emphasis for Conference

Priority	Talking point	Why it lands
1	Full digital pipeline, not isolated app screens.	Shows the project has system-level value.
2	Follow-up visibility for RUJUK and LTFU risk.	Connects technology to child health outcomes.
3	Offline-capable QR workflow for real ward conditions.	Makes the solution practical, not just idealistic.
4	Backend, RBAC, audit logs, and reports create a scalable foundation.	Shows readiness for pilot expansion and future integration.
5	Aggregate-only UNHS/MOH views protect individual baby data.	Matches governance and PDPA expectations.

What Not To Over-Emphasize

- Do not pitch it as only a dashboard. The dashboard is monitoring; the real value is the full workflow and backend pipeline.
- Do not oversell it as a complete national deployment yet. Stronger wording: pilot-ready foundation with clear scalability path.
- Do not focus only on UI. The strongest technical asset is data flow, RBAC, auditability, reporting, and integration readiness.

Future Add-On Path

Near term	Mid term	Long term
More seed data, wristbands, smoother offline sync, baby registration, pilot deployment.	SMS/WhatsApp Business reminders without transmitting clinical data, richer analytics, multi-hospital rollout.	i-Jejak/HIS integration, FHIR/HL7 interoperability, national UNHS registry and policy dashboard.

Source Notes

Malaysia context referenced from: MOH Health White Paper; DOSM ICT Use and Access by Individuals and Households Survey Report 2025; AWS announcement for Asia Pacific (Malaysia) Region; Malaysia Personal Data Protection Department page for PDPA 2010 and Personal Data Protection (Amendment) Act 2024.

Prepared as a general conference selling point brief. Core message: workflow transformation, LTFU reduction, compliance-aware governance, and scalable backend foundation.